

Measurement Guide for the National Neighbourhood Health Implementation Programme

December 2025



The
Strategy
Unit.

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1. Introduction

The **National Neighbourhood Health Implementation Programme (NNHIP)**, through extensive stakeholder engagement, has agreed a holistic suite of goals for neighbourhood working and a 'balanced scorecard' to measure outcomes. The aim is to support all Places to adopt a common approach (for within programme comparisons) and to be able to compare programme participants to non- participants (controls) in as many Places as possible.

The Programme has identified a target group for all NNHIP Places to prioritise (**adults with multiple long-term conditions (LTCs) or one LTC and identified as being at intermediate or high risk of hospital admission**) and it will assess its effectiveness, and concentrate its learning efforts, on the experience and outcomes of this group.

Over the next three years we want to be bold and collectively strive to achieve significant progress against these measures and goals to enable a long overdue shift in care from hospital to community, from sickness to prevention, from analogue to digital by creating the freedom to test, learn and support each other with developing new ways of working and generating novel solutions. The NNHIP as part of a wider Neighbourhood health (NbH) programme aims to support the following ambitions:

- Increase in PROMs and PREMs
- Increase in people's activation (confidence, skills, knowledge) to manage their LTCs
- Improvement in staff experience
- Reduction in outpatient activity
- Reduction in unplanned hospital admissions, ambulance conveyances and A&E attendances
- Improve GP access.

There is a national requirement to monitor and evaluate the impact of NNHIP interventions. Therefore, local NNHIP Places are **required to measure the progress and impact of their interventions**. This should include capturing and submitting a core set of national metrics (section 3, page 6) as well as any required local measures.

The approach to evaluation has been adapted to reflect the importance of generating evidence of impact, and the relative difficulty of showing that in this programme and timescales.



Measurement Guide for the National Neighbourhood Health Implementation Programme

This Guide has been developed to support Places to measure their delivery and impact. It is split into two broad sections. The first defines the agreed national metrics. The second defines the approach required to measure these metrics for the NNHIP cohort.

2. Summary of the overall approach

Places will need to follow these steps to measure the progress and impact of their NNHIP interventions:

- **Identify a target cohort** of eligible individuals (section 4.1, page 14)
- **(Optional step, not a programme requirement) Identify a counterfactual** with similar characteristics to your target cohort but who will not receive an NNHIP intervention (section 4.6, page 19). If identifying a counterfactual, Places should consider whether there are other neighbourhood projects outside NNHIP that may affect the counterfactual
- **Set-up data collection** for process and outcome metrics
- **Collect and monitor process metrics** for the target population cohort (i.e. the population chosen for neighbourhood interventions) and the counterfactual
- **Collect and monitor outcome metrics** for the target population cohort based on the national definitions in Table 1, page 8. If possible, outcomes should also be tracked locally at a patient level for those who receive neighbourhood health interventions
- **Submit the process and outcome metrics** for the target population cohort based to the national programme team using the submission process in section 4.8, page 19
- **Collect and monitor staff engagement metrics** for staff working within the NNHIP in neighbourhood, Place and system teams
- **Access and use the Neighbourhood Health Index** to monitor person activation, PROMs and PREMs (section 3.3, page 10)
- **Develop regular local monitoring** of all metrics.

Following engagement with Places, and reflecting the NNHIP ethos, central data collection requirements have been minimised, and national mechanisms to support Places to collect data will be introduced.

This is a learning programme. There is significant variation in approach between Places, but there will also be shared challenges. Where Places are facing difficulties in recording, reporting, information governance or otherwise adhering to the measurement guide, they are invited to share their challenges. Using the Intelligence Centre support and the experience of other Places, we hope to help overcome these issues.

3. National metrics

Each NNHIP Place is required to collect a core set of metrics to measure the progress and impact of their intervention. Places are free to collect other metrics required for local reporting but will need to collect the national metrics as a minimum. The metrics are split into four groups:

- **Process metrics** that measure the activity undertaken in each Place following the start of the programme
- **Outcome metrics** that measure the impact each Place has had on changing outcomes for patients
- **Person activation, PREMS and PROMS metrics** that measure the impact each Place has had on changing person activation, outcomes and experience
- **Staff wellbeing metrics** that measure the impact each Place has had on staff wellbeing.

Measuring the impact on health inequalities is an important aspect of the Programme. Places are required to collect and report on demographic factors for the national metrics, including:

- Gender
- Age group
- Ethnic group
- Deprivation quintile.

Places may want to collect other demographic and clinical data, such as long-term conditions, for their own reporting. This is not a national requirement.

The national metrics are defined in more detail below.

3.1 Process metrics

Process metrics measure the Place's progress with delivering their local programme. They capture the activity delivered and they are important because they provide an early indication of whether the Programme is likely to achieve its outcomes. Places can identify the most appropriate metrics for their cohort and intervention(s).

Nationally, only one process metric will be collected. This should reduce the burden on Places and ensure the reporting is meaningful. This metric is the “**number of patients on the NNHIP caseload**”. This should only include cases Places are actively working with and

not the whole NNHIP cohort. Additional qualitative indicators of maturity will be captured separately to feed into a maturity matrix for each Place. These are not included in this guide.

Given the variation in activities and interventions across NNHIP, there will be no central standardised SNOMED CT codes specific to NNHIP at this time. A list of suggested existing codes is shown in Appendix 5.

3.2 Outcome metrics

Neighbourhood working needs to ensure the effective use of resources across the system. Health and care resources are finite and NNHIP must work within the current financial restraints. Investing in services closer to people's homes should have an impact on reducing activity in other areas of health systems. Any savings from these reductions could then be redirected to fund future community capacity.

Table 1 below details the outcome metrics that must be collected by each Place. They have been chosen to be:

- Specific to the NNHIP cohort
- Based on available datasets to provide a baseline
- Linked to the programme goals.

Neighbourhood care is likely to have the most impact on acute activity, particularly reducing urgent care as clinicians and patients manage long-term conditions better. The main areas of acute urgent care that may be impacted are ambulance conveyances, A&E attendances, emergency inpatient admissions and inpatient bed days. These metrics will be collected each month by the national programme team.

Places will deliver more co-ordinated care with some planned care delivered nearer to home. This is likely to impact outpatient attendances in hospital with fewer referrals overall and a higher proportion delivered in the community. Outpatient attendances can be measured through SUS or HES using the same definition.

Better co-ordinated care should also impact on GP activity with care delivered in a more appropriate setting. The primary care measure needs to ensure it does not capture any NNHIP activity. The metric definition has been limited to routine GP consultations. Local systems should identify and consistently use specific codes for the NNHIP activity that are unique to the programme (see SNOMED CT codes in Appendix 5) and not used to record routine GP activity. These NNHIP specific codes should not be included in the outcome definition.

The outcomes that will be collected by the national programme are detailed in Table 1. The potential local considerations in the table are to support Places to develop metrics more targeted to their intervention. They do not need to be included in national submissions. More information on the definitions is included in Appendix 6 and in the submission template in Appendix 2.

Table 1: Required measures for the use of system resources

Metric	Numerator	Denominator	Potential local considerations
Number of consultant-led outpatient appointments (new, follow up and outpatient procedures) per 1,000 patients in the cohort	Number of consultant-led outpatient appointments (new, follow up and outpatient procedures) for patients in the cohort	Number of patients in the cohort (including any person who has refused treatment)	• Should the definition be limited to new appointments or include all new and follow up appointments? • Are there any specialties that should be excluded?
Number of specific acute non-elective spells in the period with a length of stay of zero days per 1,000 patients in the cohort	Number of specific acute non-elective spells in the period with a length of stay of zero days for patients in the cohort	Number of patients in the cohort (including any person who has refused treatment)	• Should all conditions be included
Number of specific acute non-elective spells in the period with a length of stay of one or more days	Number of specific acute non-elective spells in the period with a length of stay of one or more days for patients in the cohort	Number of patients in the cohort (including any person who has refused treatment)	• Should all conditions be included
Number of inpatient bed days per 1,000 patients in the cohort	Number of inpatient bed days for patients in the cohort	Number of patients in the cohort (including any person who has refused treatment)	• Should all conditions be included

Metric	Numerator	Denominator	Potential local considerations
Number of ambulance conveyances by category per 1,000 patients in the cohort	Number of ambulance conveyances by category for patients in the cohort	Number of patients in the cohort (including any person who has refused treatment)	Should all conditions be included
Number of A&E attendances (Type 1 and Other types) per 1,000 patients in the cohort	Number of A&E attendances (Type 1 and Other type) for patients in the cohort	Number of patients in the cohort (including any person who has refused treatment)	Should all conditions be included
Number of appointments in general practice and Primary Care Networks per 1,000 patients in the cohort	Number of appointments in general practice and Primary Care Networks for patients in the cohort	Number of patients in the cohort (including any person who has refused treatment)	- Should this include GP consultations only or all professionals? - Should this include consultations only or a wider definition?
Number of Community Care Contacts attended per 1,000 patients in the cohort	Number of Community Care Contacts attended for patients in the cohort	Number of patients in the cohort (including any person who has refused treatment)	Should all community contacts be included

The data collected from places for the programme will report on the above outcomes for the local target cohort. The Intelligence Centre team will also provide national tools which allow

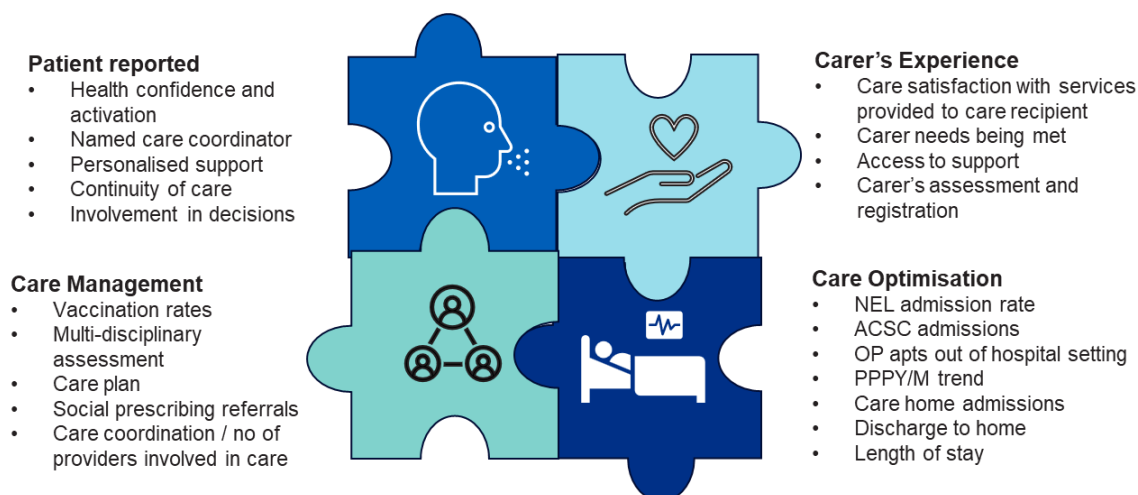
Places to track these outcomes measures for the PCNs in their footprint, but these will be for the whole PCN population, not just the target cohort.

3.3 Person activation, PREMS and PROMS: the Neighbourhood Health Index

Understanding whole person outcomes: person reported outcomes, experience and activation is a central component of Neighbourhood working targeting the right interventions to reduce emergency admissions and accelerating the cultural shift to a biopsychosocial (holistic) approach to care - recognising the deep interconnections across the system— and that value (and outcomes) come from looking at the whole picture, not isolated parts. But it is resource intensive and highly complex to establish mechanisms to measure this individually and consistently across places.

Therefore, the programme has supported the development of a centralised tool, the “Neighbourhood Index” to capture these outcomes consistently for all places and enable linkage of this data at a patient level to service utilisation, facilitating an understanding of patient journeys and impact, targeting specific interventions to improve outcomes and with minimal resource requirement from places.

Figure 1: Summary of the Neighbourhood Index



We need to ensure the **evidence for neighbourhood health and the impact of increased community capacity is attributable and defensible** and we currently have no means of consistently collecting person reported outcomes, experience and activation data and, importantly, triangulating this with activity data so we can understand the impact in the round.

The Index will build from a new neighbourhood experience and outcome survey:

- **Targeted at specific individuals** living with multiple long-term conditions identified at medium and high risk of avoidable hospitalisation and long LOS
- **Linking patient reported data, clinical codes and activity data**
- **A linked carers survey** will assess the experience and health and wellbeing of unpaid carers
- **Actionable insight for health and care professionals** to identify and follow up with individuals (where consent is provided)
- **Controlled evaluation** between intervention and non-intervention cohorts, and in time between Places participating and not participating.

We propose to **take the burden away from places** by:

- Agreeing with places a **core set of PREMs and PROMs** to be collected consistently across places (health and wellbeing measures (EQ5D), extent to which care is coordinated, personalised and accessible and transitions of care, person activation)
- Providing the **infrastructure nationally to run a survey on behalf of Places** (including communications via NHS App, SMS, email and paper)
- Securing section 251 approval from the Confidentiality Advisory Group to allow for:
 - **Places to send a list of the target cohorts** (every two to three months) to the national team to send the survey to which will enable isolation of the intervention group and save costs in circulating surveys
 - **National cohorting and sampling across a counterfactual group** identified at high and intermediate risk (we estimate this is c.600k individuals across the 43 places)
- **Providing places with a unique survey link** for them to send to local individuals they add to the cohort
- **Nationally linking person level data** from the survey to activity data to allow comparison between PREMs, PROMs and service utilisation
- **Providing each place with a dashboard** that will allow tracking of person level metrics, cohort comparison, further risk stratification, and re-id by a healthcare professional where individuals have consented.

Timescales for the Neighbourhood Index are being finalised.

Places can develop and run their own PROMs and PREMs if required. Appendix 4 provides more information to support local development. This is not required to be submitted to the national programmes team.

3.4 Staff engagement

A key goal of the programme is to enable a new way of working to improve the experience of staff and be a better way for staff to support their population. This should **improve collaborative behaviours** to deliver better outcomes to their population. These behaviours are well known to drive high performing organisations.

The **Collaborative Behaviours Assessment** provides a consistent and standardised approach. It includes a self-assessment and a collective assessment. Places have already completed a baseline assessment. The assessment is split into the following six themes.

- Theme 1 – Place Understanding
- Theme 2 – Trust & Respect
- Theme 3 – Shared Purpose
- Theme 4 – Communication
- Theme 5 – Joint Problem-Solving
- Theme 6 – Mutual Support.

This data will be collected and analysed annually. This will be done nationally with the results made available to Places.

Other surveys such as the **National Quarterly Pulse Survey (NQPS)** could be used locally to provide additional insight on staff wellbeing. This provides a consistent and standardised approach, nationally and locally, to listening to staff at more regular intervals with a robust data set. NQPS focuses on the core set of nine questions (see Table 2 below) which make up the engagement theme from the NHS Staff Survey that provide insight into motivation, involvement and advocacy.

Table 2: Engagement theme questions

Question	Rating scale
I look forward to going to work (Motivation)	• Always • Often • Sometimes • Rarely • Never
I am enthusiastic about my job (Motivation)	
Time passes quickly when I am working (Motivation)	
There are frequent opportunities for me to show initiative in my role (Involvement)	

Question	Rating scale
I am able to make suggestions to improve the work of my team/department (Involvement)	• Strongly agree • Agree • Neither agree nor disagree • Disagree • Strongly disagree
I am able to make improvements happen in my area of work (Involvement)	
Care of patients/service users is my organisation's top priority (Advocacy)	
I would recommend my organisation as a place to work (Advocacy)	
If a friend or relative needed treatment I would be happy with the standard of care provided by this organisation (Advocacy)	

Places will need to ensure the survey results can be broken to a sufficient level for them to monitor the impact of staff involved in their NNHIP intervention(s). The survey currently only covers NHS trusts. Places should explore expanding the survey to other organisations working on their NNHIP intervention(s), such as primary care, social and Voluntary, Community, and Social Enterprise (VCSE) organisations. This is not a national requirement.

Other potential workforce data includes [staff sickness rates](#) and [staff turnover rates](#) although none are specific to staff working in neighbourhoods. Places will need additional local data to make these specific their NNHIP work.

4. Measurement approach

NNHIP Places are required to collect, process and report regular process and outcome measures data. The data will need to be collected at an individual patient level and then aggregated for reporting. The following section defines the approach each Place will need to follow to **collect, process and report an NNHIP dataset**.

4.1 Cohort definition

Process and outcome data needs to be collected specifically for the NNHIP cohort. **Each Place NNHIP cohort should be defined locally** based on local priorities. The local cohort should be a subset of the national definition **adults with multiple long-term conditions (LTCs) or one LTC and identified as being at intermediate or high risk of hospital admission**. Population health management (PHM) and risk stratification tools can be used to identify the local cohort. If feasible, process and outcome metrics should be collected for the following two cohort groups.

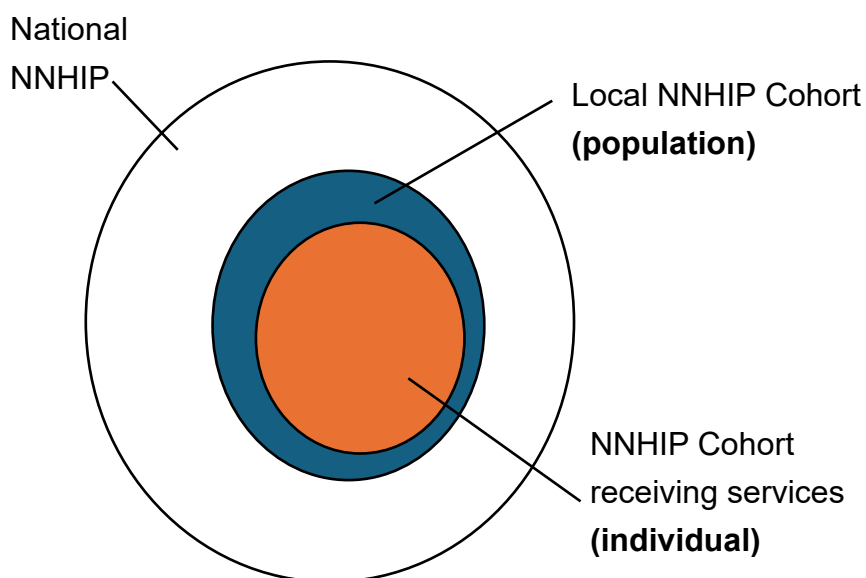
- **Population (required)** – patients identified as eligible for an NNHIP service. This should be a subset of the national cohort (see Figure 2, page 15). It will include all patients who meet the locally agreed eligibility criteria. It may include patients who do not go on to receive support from an NNHIP service.
- **Individual (if available)** – eligible patients who receive an NNHIP service. This is the subset of the population cohort that receive support from NNHIP services. New data will need to be collected on the services delivered to each person within the cohort, including the service delivery date, type of service delivered and whether the patient attended. This data will need to be linked to the outcome datasets to track the outcomes for patients who receive an NNHIP service.

Where Places do not have linked data, they will need to replicate the eligibility criteria within acute datasets. The [Hospital Frailty Risk Score \(HFRS\)](#) may be a useful proxy for the data and is readily available for Places to use. Another option would be to develop a bespoke algorithm that identifies the relevant long-term conditions and co-morbidities from your local criteria.

The NNHIP Intelligence Centre will develop and share guidance on using Population Health Management (PHM) and/or risk stratification tools to identify cohorts including support on identify the right SNOMED CT codes and how to search for them in Primary Care systems. They are also developing a new Tool to identify patient cohorts with avoidable acute activity.

Figure 2 below shows the different cohort definitions and how they relate to each other. Each one should be a subset of the higher cohort(s).

Figure 2: How the cohorts relate to each other



4.2 Recording the intervention

Places will need to record the interventions delivered to their target population. This will support monitoring process measures and more targeted outcome measures in Places with linked Primary Care and Acute datasets. Wherever possible, NNHIP neighbourhood health service clinical records should be kept in primary care GP record systems. This guidance will support Places that do not have a local data collection system in place. Where activity data is not captured in GP systems a local solution will need to be developed. Places can request support from the NNHIP Intelligence Centre if required.

The interventions and types of support offered by NNHIP integrated neighbourhood teams will be informed by the selected target population and the partners and professions involved in delivery. While there will be some variation, several interventions will be held in common (coordinated care planning, social prescribing/community connection and co-management of conditions).

To support consistent data recording and subsequent collection across NNHIP, the intelligence Centre has suggested clinical SNOMED codes that Places may want to use for their primary care clinical record keeping. These are presented in Appendix 5. Places that have already developed recording templates and codes for capturing their NNHIP activity should continue to use their locally developed systems.

4.3 Data sources

The cohort and outcomes datasets are likely to be held in different local systems. The cohort data will be collected in either primary care or community patient systems depending on the service model. Most of the outcome data is collected in the acute patient systems.

Places will need to **collect record level data on the NNHIP cohort**. This NNHIP dataset should identify who is eligible for a service and the activity undertaken by the service. It will need to include an NHS Number to support data linkage with outcome data collected in primary and acute care systems. Demographic data will also need to be collected to support analysis of health inequalities. Some of this data is likely to be recorded within clinical records, but some (for example: a list of eligible individuals) may not. Advanced planning will be required so the necessary data collections can be set-up.

The **outcomes data will already be collected locally** within primary care and acute systems. There are no routine national primary care datasets available to source the primary care outcomes. These will require local data flows to source the data for the NNHIP cohort. The national [Secondary Uses Service \(SUS\)](#) dataset can be used to source the acute system outcomes.

Places will need to collect the data items in Table 3 as a minimum to report all the required national measures for their cohort.

Table 3: Minimum data required to be collected locally by source

Data item	NNHIP dataset ¹	Primary Care	Secondary care
NHS Number	✓	✓	✓
Date of birth / age	✓		
Gender	✓		
Ethnic Category	✓		
Postcode / Lower Super Output Area (LSOA) (deprivation indicator)	✓		

¹ Collected in primary or community care depending on the service model

Data item	NNHIP dataset ¹	Primary Care	Secondary care
Risk stratification score (if used for cohort identification)	✓		
NNHIP consultation date	✓		
NNHIP consultation type	✓		
Date of primary care consultation		✓	
Type of primary care consultation		✓	
Outpatient attendance date			✓
Outpatient attended or did not attend code			✓
Outpatient attendance type (new / follow up)			✓
Outpatient treatment function code			✓
Inpatient admission date			✓
Inpatient discharge date			✓
Inpatient admission method			✓
Emergency Care Arrival Date			✓
Emergency Care Arrival Mode			✓

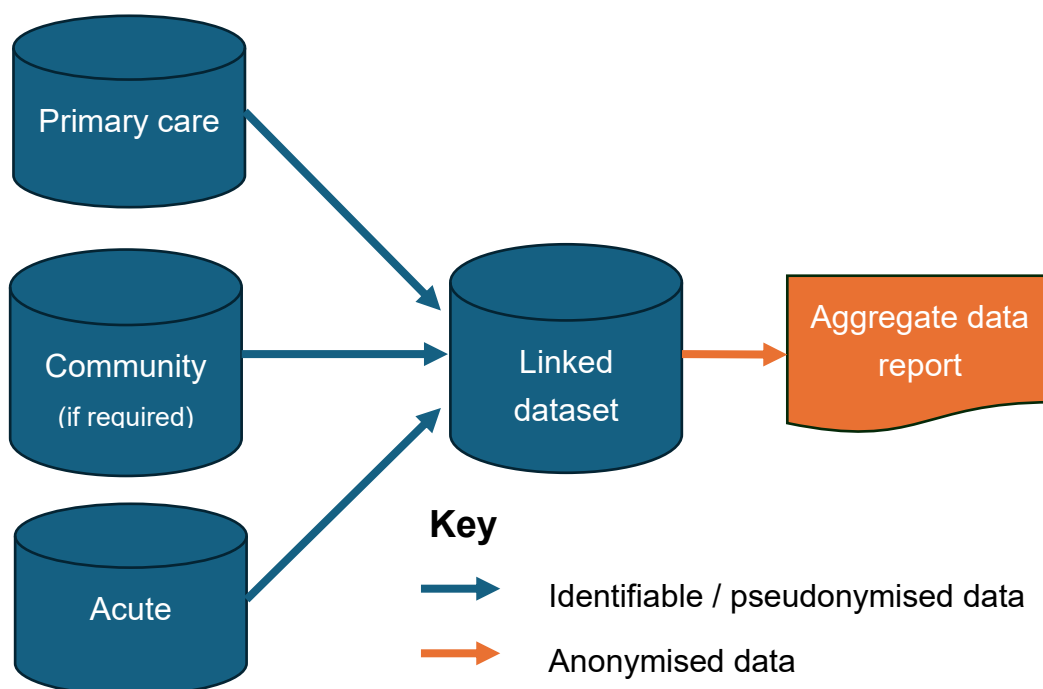
4.4 Local data collection and processing

The data required to calculate the process and outcome metrics will need to be collected in local data systems. The system will be determined locally depending on who is the main provider of NNHIP services. For example, if GPs lead the service then data would be collected in primary care systems, such as [EMIS](#) and [SystemOne](#). Additional codes may be required to capture the cohort and any activity delivered.

Data will need to be linked at an individual patient level to create the required aggregate dataset. Local data sharing agreements will be needed to link the cohort data to the outcomes (see section 4.5, page 18). This ensures only data for the relevant cohort is reported.

Figure 3 shows the required data flows for creating and reporting the aggregate dataset. Community data may not be required if the service is delivered in primary care.

Figure 3: Local data flows required to create aggregate submission



4.5 Information governance

All Places are responsible for their own information governance processes. A legal basis will be required to share and link any data. This will already be established in many Places. If there is no local data sharing, you will need to complete a Data Protection Impact Assessment (DPIA). This should detail the purpose of the data sharing, the data to be shared, the legal basis for sharing and the data sharing process. It can be used as the basis for any local Data Sharing Agreements (DSAs).

Identifying a legal basis for data sharing can be difficult. The most feasible option is a Section 251 application to the [Confidentiality Advisory Group \(CAG\)](#). Some areas already have Section 251 arrangements in place. Others may need to apply to ensure that they can legally link their process and outcomes data. Guidance on applying to the CAG can be found [here](#).

4.6 Identifying a counterfactual

Identifying a comparator group is not a requirement for the national programme. However, some Places may wish to in order to assess the impact of their implementation of neighbourhood health. Identifying a control group will help control for any confounders, such as demographic differences and other programmes aimed at the same population, and any issues with regression to the mean (see Appendix 3). Any comparator group would need to be developed locally based on the same criteria used to identify the NNHIP cohort.

The two options for developing a comparator are:

- **Matching at a population level (required)** - patients who meet the NNHIP cohort definition from within the ICB but outside the NNHIP place. Data on the process and outcome metrics will be submitted to the national programme team for this group. This could be done using propensity matching scores at Place level to control for any confounders. **Support will be available from the NNHIP Intelligence Centre** to develop matching scores, **including a guidance document and workshop for analysts**.
- **Matching at an individual level (if feasible)** - matched individual for each person receiving an NNHIP service from within the ICB but outside the NNHIP Place. For Places using risk stratification, it may be possible to match individuals on their risk score to control for any confounders. Places not using risk stratification may need to develop local propensity matching scores to control for any confounders. This is more complicated, and Places will be able to access **support from the NNHIP Intelligence Centre** to develop their matching scores.

4.7 Frequency

The data required to calculate the process and outcome metrics will be submitted, processed and reported monthly. The submission timetable is shown in Appendix 1.

4.8 Data submission and processing

Each NNHIP Place will collect and collate an aggregate dataset each month. The data will be submitted in a **standard template** (see Appendix 2) to the CSU within eight weeks of the

reporting period end. This will allow time for data lag in the outcome datasets, internal data processing and data quality checks before submission. This will be limited to:

- Aggregate data on the chosen target NNHIP caseload, including demographics
- Measures of system resource use for this cohort (Table 1, page 8).

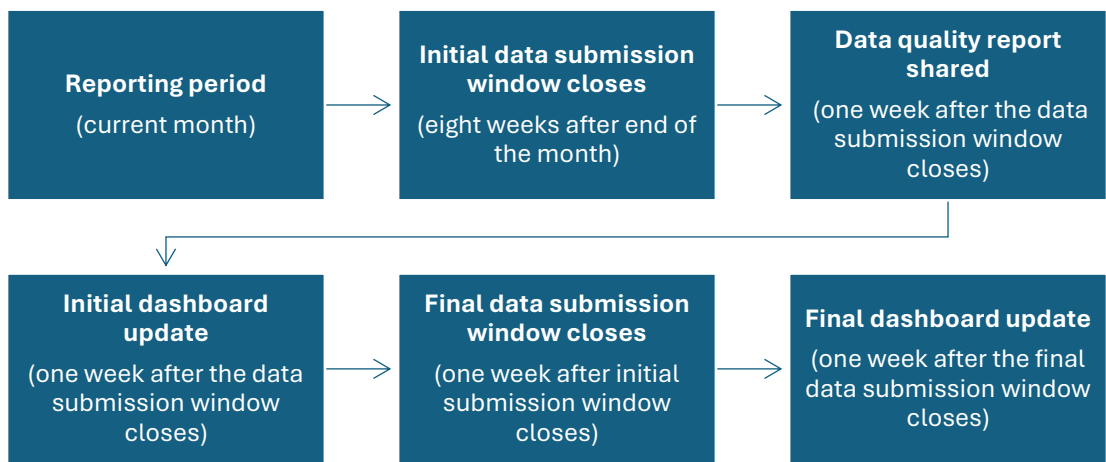
Small number suppression should be applied to the submission template. Numbers below 6 should be replaced with * to identify they have been suppressed.

The standard template will be **validated on submission** to ensure the data can be processed automatically. This will include checking the template is correct and the data is in the correct format. The initial Place level submissions will be aggregated automatically with all the other submissions for reporting.

Additional data quality checks will be run once the data has been processed. These data quality checks will be shared with Places for checking. Re-submissions will be allowed within three weeks of the initial submission period to ensure the reporting is as accurate as possible. The final validated datasets will be aggregated for all Places. These will be used for the final reporting.

The full submission process is shown in Figure 4 below.

Figure 4: Monthly data submission process with timings

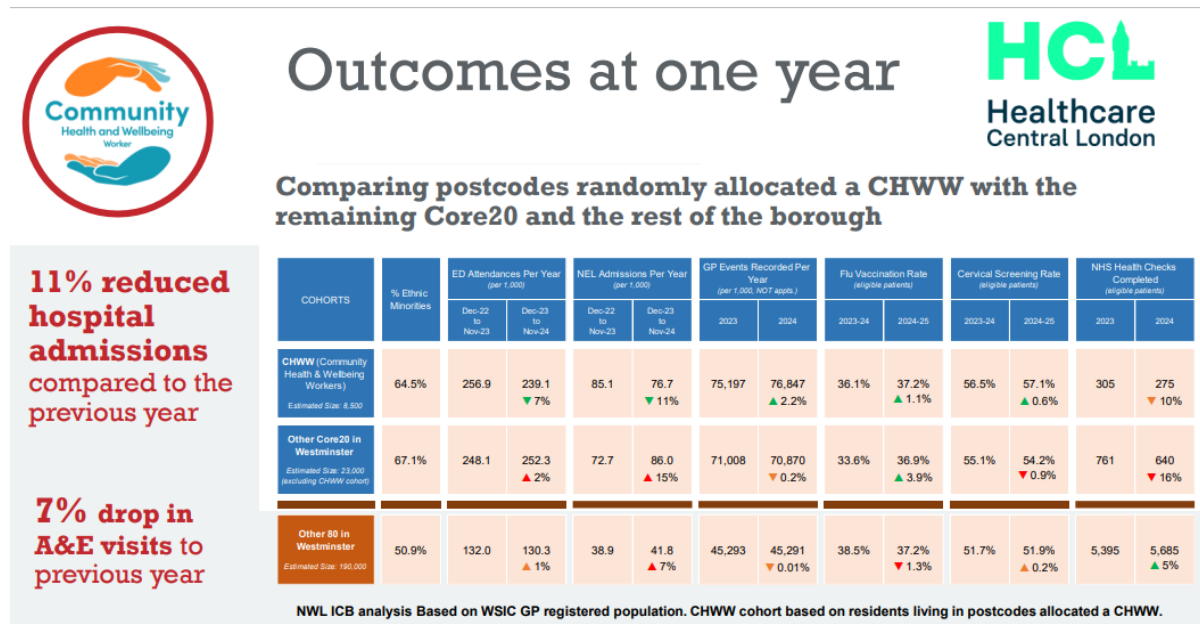


4.9 Reporting

Once processed, the data will be reported in an NNHIP dashboard. This will compare each Place pre and post NNHIP with their comparator. The data from each Place will be aggregated to show the overall national progress of NNHIP. The dashboard is currently in

development and Places will be consulted in the development. The example below shows the type of presentation being considered.

Figure 5: Example dashboard from Healthcare Central London



5. NNHIP Intelligence Centre support

The NNHIP Intelligence Centre will provide support to the national coaches and local analysts through:

- Logic model workshops with support to identify appropriate process and outcome metrics
- Guidance documents, including:
 - Evaluation guide
 - Propensity Score Matching
 - How to use PHM and/or risk stratification tools
- Workshops, including:
 - Evaluable by design
 - Developing a counterfactual and Propensity Score Matching
- Drop-in sessions
- Tools (currently being finalised) to support cohort and comparator identification.

We aim to bring together a network of analyst/evaluators/coaches from the programme who are attempting to overcome the challenges of measurement and evaluation in this complex space. Please do feedback comments, suggestions, or ideas for collaboration and peer support to england.contactneighbourhoodhealth@nhs.net, with the reference “Intelligence Centre - Evaluation”.

Appendix 1: Submission timetable

Table 4 shows the timetable for monthly submissions, including the time for data quality checks. The first submission will cover January 2026 and is due on 27th March 2026.

Table 4: Data submission timetable from January 2026 to March 2027

Month	Reporting period start	Reporting period end	Initial submission date (8 weeks)	Initial report due (1 week)	Data second submission date (1 week)	Final report date (2 weeks)
Jan-26	01/01/2026	31/01/2026	27/03/2026	03/04/2026	03/04/2026	10/04/2026
Feb-26	01/02/2026	28/02/2026	24/04/2026	01/05/2026	01/05/2026	08/05/2026
Mar-26	01/03/2026	31/03/2026	26/05/2026	02/06/2026	02/06/2026	09/06/2026
Apr-26	01/04/2026	30/04/2026	25/06/2026	02/07/2026	02/07/2026	09/07/2026
May-26	01/05/2026	31/05/2026	24/07/2026	31/07/2026	31/07/2026	07/08/2026
Jun-26	01/06/2026	30/06/2026	25/08/2026	01/09/2026	01/09/2026	08/09/2026
Jul-26	01/07/2026	31/07/2026	25/09/2026	02/10/2026	02/10/2026	09/10/2026
Aug-26	01/08/2026	31/08/2026	26/10/2026	02/11/2026	02/11/2026	09/11/2026
Sep-26	01/09/2026	30/09/2026	25/11/2026	02/12/2026	02/12/2026	09/12/2026
Oct-26	01/10/2026	31/10/2026	25/12/2026	01/01/2027	01/01/2027	08/01/2027
Nov-26	01/11/2026	30/11/2026	25/01/2027	01/02/2027	01/02/2027	08/02/2027
Dec-26	01/12/2026	31/12/2026	25/02/2027	04/03/2027	04/03/2027	11/03/2027
Jan-27	01/01/2027	31/01/2027	26/03/2027	02/04/2027	02/04/2027	09/04/2027
Feb-27	01/02/2027	28/02/2027	23/04/2027	30/04/2027	30/04/2027	07/05/2027
Mar-27	01/03/2027	31/03/2027	26/05/2027	02/06/2027	02/06/2027	09/06/2027

Appendix 2: Submission template

The template embedded below will be used to collect monthly data from each of the 43 NNHIP places.



NNHIPDataCollection
Templatev1.2_circulat

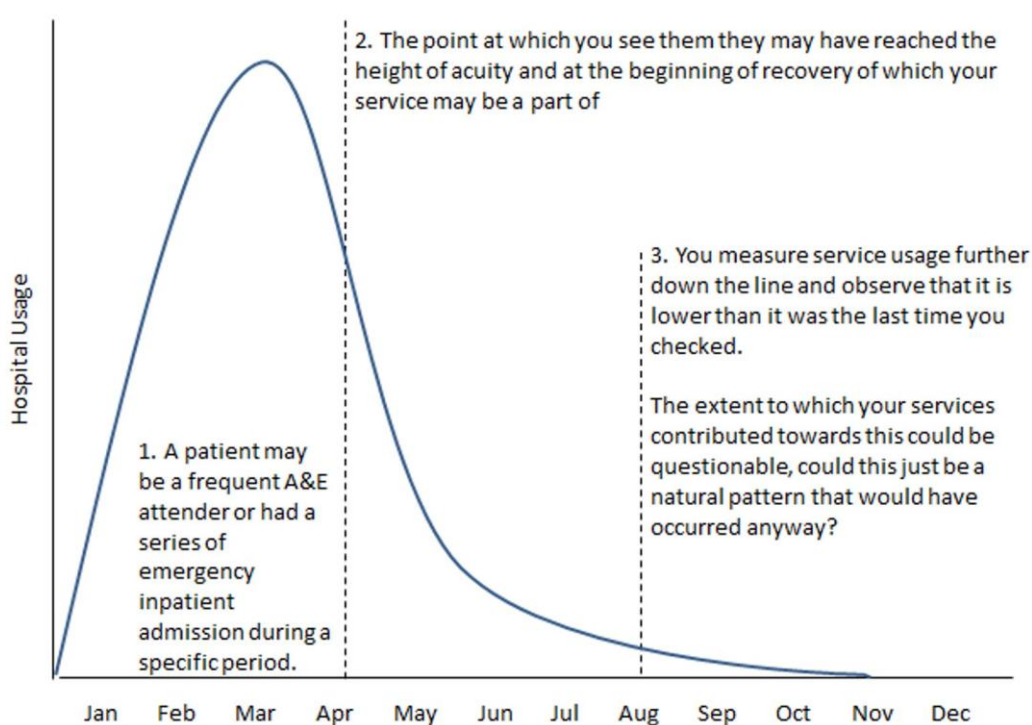
Appendix 3: Regression to the mean

The issue of 'regression to the mean' can occur whenever something which varies over time is measured once and is then measured again at a later point in time. Observations made at the extreme the first time round will tend to come back to the population average the second time round.

Regression to the mean is a particular challenge when an intervention is focused on particular types of patients (for example patients with high emergency care use). Say we look at people with frequent hospital admissions at present. On average, these individuals will have lower rates of unplanned hospital admissions in the future, even without intervention. So, if an MDT is working with patients who are currently having frequent A&E attendances, they may notice how the patients have fewer admissions over time. However, this reduction might well have occurred anyway due to regression to the mean, and it cannot necessarily be attributed to the input of the MDT. Regression to the mean occurs simply because after one extreme period, the next period is statistically likely to be less extreme.

The way to control for regression to the mean is to create a matched control group and look for differences between the intervention group and a similar control group. If you are using a before and after approach you can reflect that this may be an issue when writing up the caveats in the analysis report.

Figure 6: Regression to the mean



Appendix 4: Choice of PROMs and PREMs

Background

PROMs and PREMs provide direct evidence on the experience and effectiveness of an intervention or service from a service user perspective. They can be used alongside other data for clinical management support, monitoring and evaluation/research.

National routine collection of PROMs has only been established for surgical interventions, like hip replacement, which have clearly defined baseline and follow-up points. PROMs are more usually used in short-term evaluation or research studies. This is due to the burden of data collection, need for follow-up collection and unclear use for clinical management and monitoring purposes.

National collection of PREMs is more established with the Friends and Family test, and other patient services. These are collected as either continuously or cross-sectionally but without follow-up.

An issue across all national surveys has been a declining response rate that can impact on the validity of analysis.

The introduction of the Neighbourhood Health Index provides an opportunity for Places to gather data on patient experience and outcomes without significant local resource allocation. However, Places may have ambitions for use of these measures locally. This document provides guidance on the choice of tools for local purposes, including evaluation.

Purpose of collecting and using PROMs and PREMs

There are three main purposes for using PROMs and PREMs data – clinical management support, contract monitoring/performance management and learning / improvement / evaluation / research. The purpose will influence what data is collected, how it is collected, what engagement is required and how it is used. For example, the information governance requirements will be based on the purpose. There can be multiple purposes for use of data but these need to be specified beforehand.

Engagement

Whatever approach is used for deciding on purpose, measures and data collection there must be engagement and discussion with the Place's and staff about the expectation. This will help establish roles and responsibilities, create buy-in to any approach, and determine the practicalities of data collection. This is especially important for PROMs and PREMs given

the burden they create over routine activity metrics. Without this, the most likely result is variable up-take and limited sustainability of any data collection.

Measures

Potential PROMs for LTC

It is essential that any PROMs measure used has been formally validated. Developing a validated PROMs takes considerable time (multiple years) and resources. Therefore, for NNHIP an existing PROM is needed. Existing reviews highlight five potential PROMs measures that have some level of validation that have been used for people with multiple and/or long-term conditions:

- **LTCQ:** Assesses the impact of living with long-term conditions on quality of life, including physical, emotional, and social domains. Either 20 or 8 items versions. This measure has not been widely used or routinely applied outside validation or research studies.
- **ICECAP-A:** Measures adult wellbeing across five areas: stability, attachment, autonomy, achievement, and enjoyment. Five items in total. This measure has not been widely used or routinely applied outside validation or research studies.
- **EQ-5D:** Captures general health-related quality of life through five dimensions: mobility, self-care, usual activities, pain/discomfort, and anxiety/depression. Five questions and a VAS-scale. This is the most widely used measure and is part of NHS PROMs programme. This is due to its use in economic analysis.
- **ASCOT:** Evaluates social care-related quality of life, focusing on domains like control, personal safety, social participation, and dignity. Eight items. This measure has not been widely used or routinely applied outside validation or research studies.
- **PaRIS-PQ (Patient Questionnaire):** An internationally validated survey developed by the OECD to measure the health outcomes, care experiences, and self-management capabilities of adults aged 45 and over living with chronic conditions and receiving care in primary or ambulatory settings. Has 20-25 items cover PROM and PREM.

PROMs can be assessed for validity using the COSMIN framework ([COSMIN - Improving the selection of outcome measurement instruments](#)). An AI generated review of the above PROMs based on the COSMIN framework for use in LTC shows the following.

Table 5: AI generated review of the above PROMs based on the COSMIN framework

Measure characteristic	LTCQ	ICECAP-A	EQ-5D	ASCOT	PaRIS-PQ
Content Validity	Yes	Yes	No - generic	Yes	Yes
Structural Validity	Yes	Yes	Yes	Yes	Yes
Internal Consistency	Yes	Yes	Yes	Yes	Yes
Reliability	No	No	Yes	No	No
Measurement Error	Unknown	Unknown	Yes	Unknown	Unknown
Construct Validity	Yes	Yes	Yes	Yes	Yes
Criterion Validity	Unknown	Unknown	Yes	Unknown	Unknown
Responsiveness	No	No	Yes	No	No
Interpretability	Yes	Yes	Yes	Yes	Yes
Feasibility	Yes	Yes	Yes	Yes	Yes

While not comprehensive, the assessment flags the potential issues of responsiveness and measurement error being unclear for all the measures except EQ-5D. However, EQ-5D is not an LTC specific measure and it is unclear about its use for long-term longitudinal data collection. However, given the EQ-5D is the safe option as it is widely used and understood and is also routinely applied in NHS.

Potential PREMs for LTCs

There are four potential PREM measures. Unlike PROMs there is no established framework for assessing the validity of PREMs measures.

- Neighbourhood Index questionnaire – This is a bespoke patient survey that collects data on frail adults (based on the Hospital Frailty Risk Score (HFRS)) and can be extended to local NNHIP cohorts.
- GP patient survey – This survey could be applied. However, it is primary care focused and limited sensitivity to change. [GP Patient Survey website](#).
- Friends and family – This survey is widely used due to ease of use but provides no detailed insight into responses. [NHS England » Friends and Family Test](#)
- PaRIS-PQ – This is a combination of a number of existing PREMs surveys. [Patient-Reported Indicator Surveys | OECD](#).

Of these, the Neighbourhood Index is closest to the requirements of NNHIP.

Data collection

Accessing existing datasets

Currently there are no established national PROMs or PREMs collections that will cover the NNHIP Places and meet the objective for the data collection.

Establishing new data collections

Licences

All the measures mentioned above are free to use for non-commercial purposes. However, licences will need to be obtained, and the use registered. If additional PROMs and PREMs are identified, then the licencing for them would need to be investigated.

Measures also have guidelines on the presentation in order to ensure consistency. For example, the EQ-5D has guidelines for digital representation. This can be an issue, especially when implemented by small providers or digital versions are used.

Information Governance and approval for data collections

Unless this is a continuing use of an existing dataset with an existing purpose, then this will represent a new data collection. This will need input from IG leads in providers, ICBs, LAs and DHSC/NHSE. Time needs to be planned for this to happen, especially if it involves linking of NHS and non-NHS datasets.

If the data collection is to be nationally mandated, then the approval process for this will need to be undertaken. Experience suggests this will take approximately 12 months from inception to data being flowed.

Testing and training in data collection

The approach to data collection will need to be tested with places and amended appropriately. All staff in providers would require training in the process for data collection. This is usually best achieved with place visits or peer-led approaches, but central online training can also be provided.

Longitudinal data collection - timing and frequency of collection

The requirement is to establish a longitudinal data collection. However, there is no agreed timing, frequency or duration for data collection. If individual places use different approaches

to data collection, then this will impact on any comparative analysis that can be undertaken and likely mean that this is not possible.

Resources required for data collection

Most PROMs and PREMs are designed to be self-completed by the service user. The standard approach would be for a service users to be asked to complete the form online. Where possible this should be based on existing infrastructure, such as an existing online portal, as establishing a new data collection platform requires considerable time and resources. In addition, existing infrastructure has usually been designed for ease of use and to minimise the burden on individuals. There will also be a need for a paper version would need to be available to address digital exclusion issues.

There will still be a time commitment for staff in explaining the data collection to service users, signposting them to the data collection site and supporting them if they encounter issues.

Data quality

As with any data collection the quality of the data will be important. The most common issue with PROMs and PREMs is incomplete surveys and missing follow-up collections.

Additional data

Depending on the use of the PROMs and PREMs returns these will need to be case-mix adjusted to provide any comparative information. This will require additional information to be collected.

Risks and Issues

The primary risks for PROMs and PREMs will be low or incomplete response rates to surveys as it will impact on the validity of analysis. This can be caused by staff not making people aware of the collection, the method of data collection and people not completing the questionnaire when offered, especially over repeated time points.

There is also a risk that information governance and data collection permissions are not in place. These permissions will take time and will need input from relevant stakeholders.

Additional information

Further information on specific PROMs and PREMs

LTCQ

<https://www.hra.nhs.uk/planning-and-improving-research/application-summaries/research-summaries/living-with-long-term-conditions-validation-of-the-ltcq/>

<https://www.england.nhs.uk/personalisedcare/supported-self-management/health-system-support-framework-for-supported-self-management/examples-of-survey-tools-for-use-in-supported-self-management/>

[Long-Term Conditions Questionnaire \(LTCQ\) - Oxford University Innovation](#)

[PATIENT-REPORTED OUTCOME MEASURES FOR LONG-TERM CONDITIONS: CONCEPTUAL REVIEW AND STAKEHOLDER CONSULTATION Cheryl Hunter, Michele Peters, Anne-Sophie Darlington, Julien Forder, Angela Coulter, Crispin Jenkinson, Ray Fitzpatrick](#)

ICECAP

[ICECAP-A | Bristol Medical School: Population Health Sciences | University of Bristol](#)

EQ-5D

[User Guides - EuroQol](#)

[A comprehensive catalogue of EQ-5D scores in chronic disease: results of a systematic review - PubMed](#)

ASCOT (Person)

[Use of the adult social care outcomes toolkit \(ASCOT\) in research studies: an international scoping review | Quality of Life Research](#)

[Home - Adult Social Care Outcomes Toolkit](#)

PaRIS-PQ

[Patient-Reported Indicator Surveys | OECD](#)

[OECD PaRIS Patient Questionnaire \(PaRIS-PQ\)](#)

Additional information on application of PROMs in routine care

[Reflections on the national patient-reported outcome measures \(PROMs\) programme: Where do we go from here?](#)

[Evaluating the Population-Based Usage and Benefit of Digitally Collected Patient-Reported Outcomes and Experiences in Patients With Chronic Diseases: The PROMchronic Study Protocol - PubMed](#)

[Closing the value-based circle in shared decision-making: a digital framework for informing the shared decision-making process through patient reported outcome and experience measures - PubMed](#)

[Requirements for the collection of electronic PROMS either "in clinic" or "at home" as part of the PROMs, PREMs and Effectiveness Programme \(PPEP\) in Wales: a feasibility study using a generic PROM tool - PubMed](#)

[Implementing PROMs in routine clinical care: a qualitative exploration of GP perspectives | BJGP Open](#)

[Guidelines for the development and validation of patient-reported outcome measures: a scoping review | BMJ Evidence-Based Medicine](#)

[Systematic review of guidance for the collection and use of patient-reported outcomes in real-world evidence generation to support regulation, reimbursement and health policy | Journal of Patient-Reported Outcomes | Full Text](#)

[Self-efficacy and health-related quality of life: a cross-sectional study of primary care patients with multi-morbidity - PubMed](#)

[COSMIN - Improving the selection of outcome measurement instruments\).](#)

[NHS England » Friends and Family Test](#)

<https://www.hra.nhs.uk/planning-and-improving-research/application-summaries/research-summaries/living-with-long-term-conditions-validation-of-the-ltcq/>

[Long-Term Conditions Questionnaire \(LTCQ\) - Oxford University Innovation](#)

[PATIENT-REPORTED OUTCOME MEASURES FOR LONG-TERM CONDITIONS:
CONCEPTUAL REVIEW AND STAKEHOLDER CONSULTATION Cheryl Hunter, Michele
Peters, Anne-Sophie Darlington, Julien Forder, Angela Coulter, Crispin Jenkinson, Ray
Fitzpatrick](#)

[ICECAP-A | Bristol Medical School: Population Health Sciences | University of Bristol](#)

[User Guides - EuroQol](#)

[A comprehensive catalogue of EQ-5D scores in chronic disease: results of a systematic
review - PubMed](#)

[Use of the adult social care outcomes toolkit \(ASCOT\) in research studies: an international
scoping review | Quality of Life Research](#)

Appendix 5: Proposed Snomed CT Codes for recording NNHIP activity

The following proposed Snomed CT codes can be used by Places to capture their NNHIP activity. Not all codes will be relevant to all Places. Therefore, there is no expectation that Places collect all these codes.

NNHIP Intervention: Case management for NNHIP

SNOMED CT - Eligible for integrated care pathway (finding), SCTID: 936221000000104

SNOMED CT - On integrated care pathway (finding), SCTID: 818241000000105

SNOMED CT - Case management of long term condition (procedure), SCTID:
816791000000108

SNOMED CT - No longer on integrated care pathway (finding), SCTID: 894581000000106

SNOMED CT - Integrated care pathway declined (situation), SCTID: 877011000000107

SNOMED CT - Not on integrated care pathway (finding), SCTID: 883111000000109

NNHIP Intervention: Multi-disciplinary/integrated neighbourhood team coordinated care planning

SNOMED CT - Personalised Care and Support Planning offered (situation), SCTID:
1325721000000109

SNOMED CT - Consultation by multidisciplinary team (procedure), SCTID: 1268903003

SNOMED CT - Coordination of care team (procedure), SCTID: 464611000124102

SNOMED CT - Care planning and problem solving actions (procedure), SCTID: 225297008

SNOMED CT - Personalised Care and Support Planning declined (situation), SCTID:
1325711000000103

NNHIP Intervention: Social prescribing – community connection and community led solutions

SNOMED CT - Social prescribing offered (finding), SCTID: 871691000000100

SNOMED CT - Seen by social prescribing link worker (finding), SCTID: 1362931000000106

SNOMED CT - Social prescribing support procedures simple reference set (foundation metadata concept), SCTID: 1373511000000107

SNOMED CT - Social prescribing case closed (finding), SCTID: 1362721000000106

SNOMED CT - Social prescribing declined (situation), SCTID: 871711000000103

Interventions to support patients to co-manage their long-term conditions (for example: supported self-management, health coaching and the use of technology)

SNOMED CT - Referral to self-management support service (procedure), SCTID: 733901008

SNOMED CT - Support for self-management (regime/therapy), SCTID: 733810001

SNOMED CT - Use of digital self-management tool (observable entity), SCTID: 1129851000000104

SNOMED CT - Has self management plan (finding), SCTID: 851371000000105

SNOMED CT - Assessment of readiness for self-management (procedure), SCTID: 733863009

SNOMED CT - Does use digital self-management support tool (finding), SCTID: 1129871000000108

SNOMED CT - Provision of support for self management to caregiver (situation), SCTID: 1156887009

Appendix 6: Detailed metric definitions

Table 6 below provides detailed definitions of each metric, including the codes to identify activity where available. These are also included in the Submission Template (see appendix 2 on page 24).

Table 6: Detailed definitions of the required measures for the use of system resources

Metric	Numerator	Denominator
Number of new consultant-led outpatient appointments per 1,000 patients in the cohort	Definition: Count of outpatient appointments in the month with a MAIN SPECIALTY CODE is a consultant specific code, FIRST ATTENDANCE CODE = 1 OR 3 and ATTENDED OR DID NOT ATTEND CODE equal to 5 OR 6 and PRIMARY PROCEDURE (OPCS) IS NULL for patients in the NNHIP cohort Source: SUS Outpatients	Definition: Number of patients in the cohort (defined locally) Source: Local NNHIP data collection
Number of follow up consultant-led outpatient appointments per 1,000 patients in the cohort	Definition: Count of outpatient appointments in the month with a MAIN SPECIALTY CODE is a consultant specific code, FIRST ATTENDANCE CODE = 2 OR 4 and ATTENDED OR DID NOT ATTEND CODE equal to 5 OR 6 and PRIMARY PROCEDURE (OPCS) IS NULL for patients in the NNHIP cohort Source: SUS Outpatients	Definition: Number of patients in the cohort (defined locally) Source: Local NNHIP data collection
Number of consultant-led outpatient procedures per 1,000 patients in the cohort	Definition: Count of outpatient appointments in the month with a MAIN SPECIALTY CODE is a consultant specific code, ATTENDED OR DID NOT ATTEND CODE equal to 5 OR 6 and PRIMARY PROCEDURE (OPCS) IS NOT NULL for patients in the NNHIP cohort Source: SUS Outpatients	Definition: Number of patients in the cohort (defined locally) Source: Local NNHIP data collection

Metric	Numerator	Denominator
Number of specific acute non-elective spells in the period with a length of stay of zero days per 1,000 patients in the cohort	Definition: Count of completed inpatient spells where length of stay (Discharge Date - Admission Date) = 0 and Admission Method Code and Treatment Function Code meet the national definition set out in the AcuteNonElectiveDefs worksheet is in list for patients in the NNHIP cohort Source: SUS Inpatients	Definition: Number of patients in the cohort (defined locally) Source: Local NNHIP data collection
Number of specific acute non-elective spells in the period with a length of stay of one or more days	Definition: Count of completed inpatient spells where length of stay (Discharge Date - Admission Date) >= 1 and Admission Method Code and Treatment Function Code meet the national definition set out in the AcuteNonElectiveDefs worksheet in the submission template for patients in the NNHIP cohort Source: SUS Inpatients	Definition: Number of patients in the cohort (defined locally) Source: Local NNHIP data collection
Number of inpatient bed days per 1,000 patients in the cohort	Definition: Sum of inpatient bed days completed inpatient spells where Admission Method Code and Treatment Function Code meet the national definition set out in the AcuteNonElectiveDefs worksheet in the submission template for patients is in the cohort Source: SUS Inpatients	Definition: Number of patients in the cohort (defined locally) Source: Local NNHIP data collection
Number of category 1 ambulance conveyances per 1,000 patients in the cohort	Definition: Count of A&E attendances in the month where EMERGENCY CARE ARRIVAL MODE IN (1048051000000107, 1048031000000100, 1048041000000109) and EMERGENCY CARE ACUITY = '1064891000000107' for patients in the NNHIP cohort Source: ECDS	Definition: Number of patients in the cohort (defined locally) Source: Local NNHIP data collection
Number of category 2 ambulance conveyances per 1,000 patients in the cohort	Definition: Count of A&E attendances in the month where EMERGENCY CARE ARRIVAL MODE IN (1048051000000107, 1048031000000100, 1048041000000109) and EMERGENCY CARE ACUITY = '1064911000000105' for patients in the NNHIP cohort Source: ECDS	Definition: Number of patients in the cohort (defined locally) Source: Local NNHIP data collection

Metric	Numerator	Denominator
Number of category 3 ambulance conveyances per 1,000 patients in the cohort	Definition: Count of A&E attendances in the month where EMERGENCY CARE ARRIVAL MODE IN (1048051000000107, 1048031000000100, 1048041000000109) and EMERGENCY CARE ACUITY = '1064901000000108' for patients in the NNHIP cohort Source: ECDS	Definition: Number of patients in the cohort (defined locally) Source: Local NNHIP data collection
Number of category 4 ambulance conveyances per 1,000 patients in the cohort	Definition: Count of A&E attendances in the month where EMERGENCY CARE ARRIVAL MODE IN (1048051000000107, 1048031000000100, 1048041000000109) and EMERGENCY CARE ACUITY = '1077241000000103' for patients in the NNHIP cohort Source: ECDS	Definition: Number of patients in the cohort (defined locally) Source: Local NNHIP data collection
Number of A&E attendances at a type 1 department per 1,000 patients in the cohort	Definition: Count of A&E attendances in the month where EMERGENCY CARE DEPARTMENT TYPE = 01 for patients in the NNHIP cohort Source: ECDS	Definition: Number of patients in the cohort (defined locally) Source: Local NNHIP data collection
Number of A&E attendances at an other type department per 1,000 patients in the cohort	Definition: Count of A&E attendances in the month where EMERGENCY CARE DEPARTMENT TYPE <> 01 for patients in the NNHIP cohort Source: ECDS	Definition: Number of patients in the cohort (defined locally) Source: Local NNHIP data collection
Number of appointments in general practice and Primary Care Networks per 1,000 patients in the cohort	Definition: Count of general practice and PCN appointments where Appointment Status = 'Attended', SDS Role Group = 'GP' and Service Setting in ('General Practice','Primary Care Network') for patients in the NNHIP cohort Source: Local primary care systems	Definition: Number of patients in the cohort (defined locally) Source: Local NNHIP data collection
Number of Community Care Contacts attended per 1,000 patients in the cohort	Definition: Count of care contacts where cyp201carecontact.Unique_MonthID = 'Unique_MonthID' and cyp201carecontact.Contact_Date between 'ReportingStartDate' and 'ReportingEndDate' for patients in the NNHIP cohort Source: CSDS	Definition: Number of patients in the cohort (defined locally) Source: Local NNHIP data collection

Metric	Numerator	Denominator
Number of patients on the NNHIP caseload	Definition: Count of active cases the Place is actively working with during the month. This should not be the whole NNHIP cohort. Source: Local NNHIP data collection	